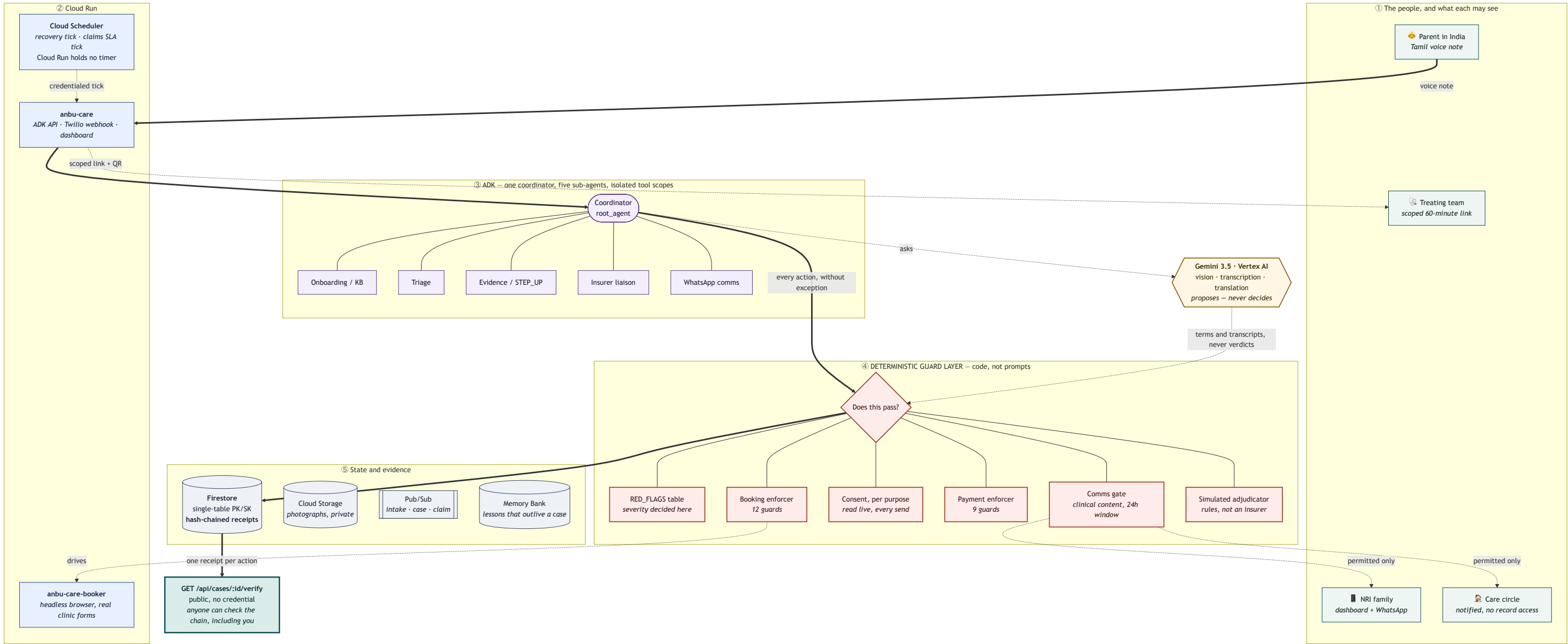


Anbu Care

Agents propose, guards decide, the chain records. Band 4 is drawn as its own layer on purpose, and the request spine runs straight through it. Everything in that band is deterministic code the model cannot reach, argue with, or widen. That separation is the architecture: a diagram that mixed the two would be describing a different system.



What is managed, and what it refuses

Google-managed, and the command that proves it

A pillar with no check beside it is a pillar somebody has to take on trust. \$URL is <https://anbu-care-37j4eofpwq-el.a.run.app>

Cloud Run	Agent API, Twilio webhook, dashboard. A second service runs the headless browser: Chromium will not fit beside the API.	<pre>gcloud run services list --region asia-south1</pre>
Firestore	Case state and the hash-chained receipt ledger, single-table PK/SK.	<pre>curl -s \$URL/api/cases/case-dalc2cb6db/verify</pre>
Cloud Scheduler	The recovery tick and the claims SLA tick. Cloud Run holds no timer, which is what makes the regulatory clocks real.	<pre>gcloud scheduler jobs list --location asia-south1</pre>
Pub/Sub	Intake, case and claim events across a multi-day admission.	<pre>gcloud pubsub topics list</pre>
Cloud Storage	Photographs, booking screenshots, claim forms. Private, reached only through signed URLs that expire.	<pre>curl -o /dev/null -w '%{http_code}' \$URL/api/parents/ -> 401</pre>
Vertex AI	Gemini 3.5 Flash for the agent fleet, document vision, Tamil transcription and translation. Gemini 2.5 Flash Lite for one question with a two-letter answer.	<pre>curl -s \$URL/api/healthz</pre>
Agent Engine Memory Bank	The one store that outlives a case. Recall is an exact scope lookup, never a similarity search.	<pre>ANBU_MEMORY_BANK_LIVE=... pytest -m memory_bank</pre>
Google Places	Every hospital and diagnostic centre carries a place_id and a verification date, so distance is real.	<pre>Shown on every triage call, with the seed date</pre>

Verification is public. GET `/api/cases/<id>/verify` needs no credential, because it proves the record was not altered without revealing what it says. Everything that returns content is credentialed. **1,221 tests**, none needing GCP or a model. **Simulated:** the insurer’s adjudicator, and payments run on a real Razorpay link in test mode. **Not simulated:** WhatsApp, the clinic booking, the regulatory clocks, hospital locations, and the receipt chain.

What this system will not do

Each of these is enforced in code rather than asked for in a prompt, and each has a test that fails if the guard is removed.

Severity is never argued down	“She says it’s probably just gas” still returns HIGH. The table that decides severity is code, and never reads that sentence as permission.
Clinical detail never leaves over WhatsApp	The gate classifies the content, not the caller’s claim about it. Bypass the agent and call send() directly: still blocked.
It will not attribute a result it cannot place	Two tests outstanding and one report arriving closes neither. Deciding which would be a model choosing which clinical order was carried out.
It will not pay the insurer’s share	Under cashless the insurer settles with the hospital, so only the family’s residual is paid. INR 27,300 on the paper, INR 9,733 owed.
It will not claim an action it did not take	A booking is recorded as requested, never confirmed, because an unauthenticated callback form cannot truthfully produce more.
It will not put a sentence in memory	Each lesson has its own function composing its own sentence from a validated value. A caller cannot store a symptom because a caller cannot store prose.
It will not guess on a form somebody signs	The Part A claim form prints “not on record” rather than a plausible value, and is left unsigned.
It does not watch anyone	No sensors, no monitoring. An episode begins because a signal arrives, and the receipt says so. Tests reject the word “detect” in that path.